



PROCEDURE

Dysphagia Screening

Scope (Staff):	NMHS Mental Health (MH): Clinical Staff
Scope (Area):	NMHS Mental Health (MH) Areas: Inpatient Services

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1. Aim

The aim of the Dysphagia Screening Procedure is to minimise the risk of aspiration pneumonia, choking, malnutrition, dehydration and other adverse outcomes.

This procedure establishes minimum practice standards for Registered Nurses performing dysphagia screening in accordance with the [MH Dysphagia Management Policy](#) by;

- Identifying patients with swallowing difficulties, or who are *at risk* of swallowing difficulties and/or aspiration, and
- Minimising unnecessary nil by mouth (NBM) status by using an endorsed screening tool.

2. Background

The following documents are required to give effect to this procedure:

- [DoH WA Clinical Handover Policy](#)
- [DoH WA Clinical Handover Guideline](#)
- [DoH Consent to Treatment Policy](#)
- [MHPHDS Interpreter Services and Translated Information Policy](#)
- [MH Management of Clinical Alerts Policy](#)

3. Risk

Non-compliance with this procedure will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☒	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☒	Research & Innovation	☐
Information Communications Technology	☒	Staff Safety & Wellness	☐
Facility Management	☐	Workforce Planning	☒
Medical Equipment	☐	Other	☐

4. Definitions

The following definitions are relevant to this policy:

Aspiration pneumonia: is an infection of the lungs that can occur when the swallowing process fails and a substance (fluid, food, medicines, saliva, vomit or small foreign object) is inhaled into the lungs. It is a serious health condition which can be fatal.

Carer: is a person who (without being paid) provides ongoing care or assistance to another person who has a disability, a chronic illness or a mental illness, or who is frail and aged.

Clinical team: The clinical team includes all health professionals participating in the delivery of care at all stages of an episode of care.

Clinician: A person, registered under the *Health Practitioner Regulation National Law (WA) 2010*, mainly involved in the area of clinical practice that includes the diagnosis, care and

treatment, including recommended preventative action, to patients. Clinicians include nurses, midwives, medical officers and allied health professionals.

Clinical handover: Clinical handover is an explicit transfer of information supporting the transfer of clinical accountability and responsibility between healthcare professionals to enable continuity of care for the patient. It refers to any situation in which professional responsibility and accountability for some or all aspects of care for a patient, or group of patients, is transferred to another person or professional group on a temporary or permanent basis.

Clinical handover framework: Refers to the structures and processes identified by the DoH to achieve a consistent and standardised approach to inform clinical handovers undertaken by clinicians within WA health services.

Community health services: Community health services refer to services for clients that are provided by Health Service Providers (and Contracted Health Entities) outside the hospital setting.

Dysphagia Swallow Screen: A Dysphagia Swallow Screen (DSS) is endorsed by the service and for use by nurses trained in the use of the DSS for patients on modified diets during meals (B).

Dysphagia: Dysphagia is the term used to describe difficulty swallowing food, fluid or saliva at the oral, pharyngeal and/or esophageal stage

Dysphagia Screening: is a process to identify patients experiencing difficulties in swallowing. Dysphagia screening may be undertaken by a Registered Nurse competent in the use of the endorsed Dysphagia Screening Tool (DST).

Dysphagia Screening Tool: Dysphagia Screening Tool (DST) endorsed to guide Registered nurses in the identification and management of patients at risk of dysphagia /aspiration (**Appendix 1**). The tool is derived from the *DST developed at the Royal Brisbane Woman's Hospital*.

Dysphagia Assessment: is a process undertaken by a Speech Pathologist to identify specific difficulties with swallowing (drinking, chewing, eating, controlling saliva, taking medication, protecting the lungs from food or drink).

iSOBAR: iSOBAR refers to the **6 key steps** in the clinical handover process that specify minimum requirements for safe and effective clinical handover; **I**dentify; **S**ituation; **O**bservations; **A**greed Plan / **A**ssessment and **R**eadback/**R**isk/**R**ecommendation (adapted from the WA Health Clinical Handover Policy).

Person of Concern: A person that a clinician is particularly concerned about, as defined by the treating clinician ([MH Person of Concern Protocol](#)). This includes a person admitted to a mental health facility in the previous twenty four hours, a person with identified clinical risks, a person who had a medical or mental health emergency in the previous twenty four hours, a person being monitored for mental or physical deterioration, a person discharged from a mental health facility referred to Community MHS and any other person of clinical concern ([WA DoH Health Clinical Handover Policy](#)).

Swallowing: (deglutition) is the process that allows for a substance (fluid, food, medicines, saliva) to pass from the oral cavity, to the pharynx, and into the oesophagus, while shutting the epiglottis to assist the movement of fluid via the oesophagus to the stomach.



Swallow Reflex: The automatic temporary closing of the epiglottis is controlled by the swallowing reflex and when this function is impaired the epiglottis remains open and food, fluid or other foreign material is inhaled, increasing the risk of an infection of the lungs (i.e. aspiration pneumonia).

Swallow Screen: is undertaken as a component of Dysphagia Screening and assists in identifying fluids and foods that are appropriate for the individual pending completion of a comprehensive Dysphagia Assessment.

5. Key Points

Screening for dysphagia using the Dysphagia Screening Tool (DST) must only be undertaken by a Registered Nurse that has completed training and is deemed competent in undertaking the DST.

6. Equipment

- Dysphagia Screening Tool (PMR14A) (**Appendix 1**)
- 80mL water or patient's current level of thickened fluid
- Suction Machine

7. Dysphagia Screening Procedures and Processes

7.1 Pre-Screening

Dysphagia screening is to be performed where suction equipment is available and functioning.

- Ensure patient is alert and cooperative.
- Patient identification and procedure matching processes are undertaken according to NMHS Patient Identification Policy.
- Screening procedure is explained to patient and family/caregiver, and appropriate consent is given as per [NMHS Consent to Treatment Policy](#).
- Provide the opportunity for an accredited interpreter where appropriate to the patient's language or communication requirements – refer to the [MHPHDS Interpreter and Language Services HealthPoint Page](#).
- Patient privacy and dignity is maintained, and distractions are removed.
- Ensure patient is seated in an upright position, as clinically indicated. Consult with relevant members of the team if there are any positioning restrictions.
- Assess patient for mouth care and undertake as required.
- Hand hygiene must be carried out as per [MHPHDS IPC Hand Hygiene Policy](#).

7.2 Screening using the DST

The nurse undertakes the Dysphagia Screening Tool (**Appendix 1**) for patients at risk of aspiration as identified in the Dysphagia Symptomology Checklist (**Appendix: 2**) to identify any swallowing difficulties and implement an interim management plan including dietary restrictions pending assessment by the Speech Therapist (Refer to **Appendix 6: Speech Pathology Care Plan and Report** (PMR14)).



7.3 Post Screening

Following completion of the DST clinicians are to follow the management recommendations described on the DST.

7.4 Patients who are identified as being at risk of dysphagia or aspiration

Clinicians follow the management recommendations as recommended using usual escalation and communication processes as follows:

Notify senior nurse

Notify medical officer to determine

- if patient is to be placed on a standby diet in accordance with the International Dysphagia Diet Standardisation Initiative (IDDSI) Level 3 moderately thick fluids and Level 4 pureed diet) requiring full nursing supervision during meals or
- if patient is to be **NBM** pending an assessment by the Speech pathologist.

A. For patients identified as requiring NBM consider the following options:

- Commencing IV fluids
- Commencing NG feeds
- Referral to Speech Pathology for dysphagia assessment
- Referral to Dietetics to ensure nutrition and hydration requirements
- Transfer to a General Hospital (via Ambulance)

B. For patients identified as requiring a stand-by diet

- Stand-By diet ordered- IDDSI **Level 3** moderately thick fluids and **Level 4** puree diet
- Patients placed on a stand-by diet MUST be assessed at each meal using Dysphagia Swallow Screen (**Appendix 3**)
- Nursing staff MUST provide full supervision during oral intake, including eating, drinking and taking medications. always remain with the patient when taking fluids or food
- Nursing staff must monitor the swallow of all patients who have commenced oral intake using the Dysphagia Swallow Screen during each meal (**Appendix 3**)
- Referral to Speech Pathology for dysphagia assessment
- Referral to Dietitian to ensure nutrition and hydration requirements

7.5 The Dysphagia Swallow Screen

The Dysphagia Swallow Screen is to be undertaken with each meal and based on the outcome of the Swallow Screen the patient may be required to cease all Oral Intake and be NBM until further medical review for consideration of IV Fluids or NG feeds (**Appendix 3**).

- Referral to Speech Pathology for dysphagia assessment
- Referral to Dietitian to ensure nutrition and hydration requirements

7.6 Additional Requirements

- Patient to remain in an upright position for a minimum of 30 minutes as able to be tolerated.
- 2 hourly oral care is required for patients that are NBM
- Regular oral care for all patients with risk of dysphagia
- Notify CNS/shift coordinator re: outcome of screen
- Update iSoft Clinical Manager (iCM) – Nursing Handover 'diet/diet notes
- Record outcome and actions in clinical notes
- Record outcome and actions in Psolis
- Inform catering staff of any dietary changes

7.7 Risk Alerts

- Patients assessed as having dysphagia must have a Dysphagia Risk Alert Sticker in the medical record (**Appendix 4**).
- Clinical Alert Form is completed and placed at the front of the medical record (**Appendix 5** Clinical Alert Form PMR01).
- Risk Alert is entered in accordance with the [MH Management of Clinical Alerts Policy](#).

7.8 Patients who are identified as NOT being at risk of dysphagia or aspiration

Place patients on their usual diet and fluids (ensure records/handovers are checked to ensure latest diet and fluids recommendations) and inform catering.

8. All Clinical Handovers

- Patients, carers and family members are included in clinical handover, where appropriate.
- Accountability for care of a patient needs to be defined at clinical handover by members of the receiving clinical team.
- Patients of concern, as identified by one of their treating clinicians, must be prioritised for clinical handover ([MH Person of Concern Protocol](#))
- All clinical handovers have a consistent structure and content, such as the iSoBAR format below;

i	IDENTIFY	Introduce yourself and your patients
S	SITUATION	Describe the reason for handing over
O	OBSERVATIONS	Include vital signs and assessments
B	BACKGROUND	Pertinent patient information
A	AGREE A PLAN	Given the situation, what needs to happen
R	READ BACK RISK RECOMMENDATION	Confirm shared understanding

9. Professional Education and Training

Profession Education and Training (PEaT) coordinates the following activities:

Identified Registered Nurses must complete the MH e-learning module for Dysphagia Screening Tool (DST) and competency training in the use of the screening tool (PEaT training module).

Nursing staff complete eLearning module and training relating to dysphagia and the use of DSS

Nursing staff attend annual training/refresher in accordance with [MH Recognising and Responding to Acute Deterioration Policy](#) that includes Basic Life Support, airway management, early recognition and management of Sepsis (Refer to the Sepsis Pathway in the [MH Recognising and Responding to Acute Deterioration Policy](#) and [MHPHDS Mandatory Training Policy](#)).

10. Compliance and Evaluation

- 100% of patients with indicators of dysphagia / aspiration risk are screened using the DST on admission (documentation audit).
- 100% of dysphagia risk alerts are entered in the patient medical record and into PSOLIS within 24 hours of identification of risk (documentation audit).
- 100% of Registered Nurses undertaking the DST are deemed competent
- 100% of nurses (RN/EN) undertaking the DSS have completed training
- MHPHDS Safety, Quality and Performance report on audit and performance data to the Mental Health Clinical Governance Committee and MHPHDS Executive ([SQP Audit tools](#))

Related internal policies, procedures and guidelines

[MHPHDS Mandatory Training Policy](#)
[MH Management of Clinical Alerts Policy](#)
[NMHS Clinical Documentation Policy](#)
[MH Recognising and Responding to Acute Deterioration Policy](#)
[MH Clinical Risk Assessment and Management Policy](#)
[MH Physical Healthcare Management Policy](#)
[MH Role of the Allocated Nurse in Inpatient Services Procedure](#)
[MH Person of Concern Protocol](#)
[MHPHDS IPC Hand Hygiene Policy](#)
[NMHS Clinical Documentation Policy](#) (contains links to WA Clinical Handover Framework)
[MHPHDS Interpreter Services and Translated Information Policy](#)
[DoH Statewide Standardised Clinical Documentation \(SSCD\) Triage to Discharge Framework](#)

References

[WA DOH Clinical Handover Policy](#)
[WA DoH Clinical Handover Guideline](#)
[WA Clinical Incident Management Policy](#)
[ACSQHC Standard 8 Recognising and Responding to Acute Deterioration](#)
[MP 0086/18 Recognising and Responding to Acute Deterioration Policy](#)
[WA DoH Recognising and Responding to Acute Deterioration Guideline](#)

Useful resources (including related forms)

[Stroke Foundation](#). (2017). *Clinical guidelines for stroke management 2017*. Melbourne, Australia.
[Royal Brisbane Women's Hospital Dysphagia Screening Tool \(RBWH DST\), 2006](#)
International Dysphagia Diet Standardisation Initiative (IDDSI) www.iddsi.org
MH Dysphagia Screening Tool – PMR14A
MH Dysphagia Swallow Screen Chart – PMR14B

Policy Sponsor	Director of Nursing MHPHDS				
Policy Contact	Director of Nursing MHPHDS				
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Approved by:	Director of Nursing MHPHDS			Date:	28/07/2022
Endorsed by:	Director of Clinical Services MHPHDS			Date:	29/07/2022
NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☐  Std 2: Partnering with Consumers ☒  Std 3: Preventing and Controlling Healthcare Associated Infection ☐  Std 4: Medication Safety		☒  Std 5: Comprehensive Care ☒  Std 6: Communicating for Safety ☐  Std 7: Blood Management ☒  Std 8: Recognising and Responding to Acute Deterioration		
National Standards for Mental Health Services	☒ Std 1: Rights and Responsibilities ☒ Std 2: Safety ☒ Std 3: Consumer and Carer Participation ☐ Std 4: Diversity Responsibility ☒ Std 5: Promotion and Prevention ☒ Std 6: Consumers ☒ Std 7: Carers ☐ Std 8: Governance, leadership and management		☐ Std 9: Integration ☒ Std 10: Delivery of Care ☐ 10.1 Supporting Recovery ☐ 10.2 Access ☐ 10.3 Entry ☐ 10.4 Assessment and Review ☐ 10.5 Treatment and Support ☐ 10.6 Exit and Re-entry		
Chief Psychiatrist's Standards for Clinical Care:	Aboriginal Practice, Assessment, Care Planning, Consumer and Carer Involvement in Individual Care, Physical Health Care of Mental Health Consumers, Risk Assessment and Management, Transfer of Care				
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Document Version Control			
Date	Version	Author	Notes
29/07/2022	1.0	Director of Nursing MHPHDS	Policy to identify the mandatory requirements for the management of Dysphagia in inpatient MHS by the multidisciplinary team
09/03/2023	1.2	Policy and Audit Officer	Endorsed Dysphagia Screening Tool PMR14A and Dysphagia Swallow Screen Forms PMR14B added

The health impact upon Aboriginal people have been considered, and where relevant incorporated and appropriately addressed in the development of this health initiative (**ISD No. 1545**).

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Appendix 1: PMR14A Dysphagia Screening Tool (DST) page 1

NORTH METROPOLITAN HEALTH SERVICE MENTAL HEALTH Hospital MH DYSPHAGIA SCREENING TOOL (DST) <i>Only to be used by trained clinical staff</i> For Patients with Dysphagia and/or Aspiration Risk		Surname Sex U.R. No Forenames D.O.B. Address Ward Registrar Consultant Use Patient I.D. Label when available
Phase 1 Screening Procedure Does patient have any of the following or previous history of the following? (tick if applicable)		
Dysphagia or aspiration risk on previous admission(s)		☐ Yes ☐ No
COPD +/- upper GI disorder		☐ Yes ☐ No
Stroke		☐ Yes ☐ No
Neurological involvement		☐ Yes ☐ No
Head injury		☐ Yes ☐ No
Head & neck cancer +/- surgery / chemotherapy / radiation		☐ Yes ☐ No
Orthopaedic conditions – # NOF, cervical spinal surgery		☐ Yes ☐ No
Acutely unwell, frail aged with co-morbidities		☐ Yes ☐ No
Suspected aspiration pneumonia / recurrent chest infections		☐ Yes ☐ No
Severe disability (e.g. physical disability)		☐ Yes ☐ No
– If 'Yes' to any of the above boxes proceed to Phase 2. If all boxes are ticked 'No', patient to have usual modified diet or normal diet & thin fluids. Complete 'Swallowing Management Plan' below. Dysphagia Swallow Screen (DSS) Initiated		
Phase 2 Interview with Patient / Family / Carer (tick if applicable or assessable)		
Do you have difficulty eating food / drinking fluid?		☐ Yes ☐ No
Do you cough or have choking episodes whilst eating / drinking?		☐ Yes ☐ No
Do you need to modify the food you eat? e.g. softer or smaller pieces		☐ Yes ☐ No
If 'Yes', provide details re current diet / fluid consistency: Diet: Fluid:		
Do you get chest infections?		☐ Yes ☐ No
If 'Yes', Is this recent? Yes ☐ No ☐		Has it been investigated before? ☐ Yes ☐ No
Indicators of Dysphagia / Aspiration Risk (tick if applicable)		
Altered LOC / reduced responsiveness		☐ Yes ☐ No
Respiratory rate >30bpm		☐ Yes ☐ No
Slurred speech (dysarthria)		☐ Yes ☐ No
Weak / absent volitional cough		☐ Yes ☐ No
Not managing oral secretions (drooling, wet voice)		☐ Yes ☐ No
Weak voice (dysphonia)		☐ Yes ☐ No
Suspected aspiration pneumonia / recurrent chest infections		☐ Yes ☐ No
Documented or history of dysphagia / aspiration risk		☐ Yes ☐ No
Reports difficulty swallowing		☐ Yes ☐ No
Reported coughing / choking episodes whilst eating / drinking		☐ Yes ☐ No
Water Test: Administer only if 'no' has been ticked for all indicators of dysphagia. Give the patient 85mls (1/3 cup) water.		
Coughing during or between swallows, or up to one minute after swallowing		☐ Yes ☐ No
Wet / gurgly or hoarse voice after swallowing		☐ Yes ☐ No
Increased respiratory rate after swallowing		☐ Yes ☐ No
If 'Yes' to any of the above Indicators or the Water Test, medical officer must be advised. Patient to remain NBM. Refer to SP. If SP is NOT available, MO to determine hydration / nutrition / medication options e.g. NBM, IV fluids / NG feeds OR 900mls Thickened Fluid and Smooth Puree Diet. If commenced on oral fluids / diet Dysphagia Swallow Screen to be undertaken at every meal. If 'No' to all of the above Indicators and Water Test, place patient on usual modified diet or normal diet & thin fluids. DSS Initiated.		
Swallowing Management Plan		
Patient identified as dysphagic or aspiration risk. ☐ Yes ☐ No		Notify the Medical Officer. ☐ Yes ☐ No
If 'Yes': Patient placed NBM: ☐ Yes ☐ No		IV fluids: ☐ Yes ☐ No
900mls Thickened Fluid and Smooth Puree Diet.		NG feeds: ☐ Yes ☐ No
Refer to Speech Pathologist		If Yes initiate DSS
		Date referred:
If 'No': ☐ Patient placed on general diet and thin fluids; ☐ OR usual modified: ☐ Diet Fluids:		
Print Name: Designation: Signature:		
Ward: Unit: Date: Time:		

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02/23

Adapted from RBWH framework for use in MHS

PMR14A MH DYSPHAGIA SCREENING TOOL (DST)

Appendix 1: PMR14A Dysphagia Screening Tool (Flow Chart) page 2

NORTH METROPOLITAN HEALTH SERVICE MENTAL HEALTH Hospital	<table border="1" style="width: 100%; border-collapse: collapse;"> <tr> <td style="width: 50%;">Surname</td> <td style="width: 10%;">Sex</td> <td style="width: 40%;">U.R. No</td> </tr> <tr> <td colspan="2">Forenames</td> <td>D.O.B.</td> </tr> <tr> <td colspan="3">Address</td> </tr> <tr> <td>Ward</td> <td>Registrar</td> <td>Consultant</td> </tr> <tr> <td colspan="3">Use Patient I.D. Label when available</td> </tr> </table>	Surname	Sex	U.R. No	Forenames		D.O.B.	Address			Ward	Registrar	Consultant	Use Patient I.D. Label when available		
Surname	Sex	U.R. No														
Forenames		D.O.B.														
Address																
Ward	Registrar	Consultant														
Use Patient I.D. Label when available																

Assessment and Follow-Up Plan

Phase 1:
Conditions commonly linked to dysphagia
Does the patient have any of the conditions linked to dysphagia outlined on screening tool?

YES

Phase 2: Patient / Carer Interview and Indicators of dysphagia. Does patient have any indicators of dysphagia outlined in Phase 2 of screen?

YES

Refer to Speech Pathologist. **NBN** until reviewed by Medical Officer to determine if NBN, IV Fluids, NG feeds, thickened fluids required.

NO

Proceed to Water Test.
Are there any indicators of dysphagia when the patient drinks 85mls of water?

YES

Swallowing Management Plan: Advise Medical Officer. The patient must remain **NBM** and referred to Speech Pathologist. If Speech Pathologist is not available, the Medical Officer to determine nutrition / hydration medication options.

↓

Complete Swallow Management Plan.
Document in MR.
Include in Clinical Handover.

NO

Patient may commence usual modified diet or normal diet. Refer to Management Plan. **Dysphagia Swallow Screen** initiated for ongoing monitoring.

Appendix 2: Dysphagia Symptomology Checklist

Evidence on Food and Fluid Intake

General:

- Difficulties managing oral secretions or drooling
- Absence or weakness of a voluntary cough or swallow
- Changes in voice quality/ tone (hoarseness/ moist sounding)
- Decreased mouth and tongue movements
- Tongue thrust/ primitive oral reflexes
- Frequent throat clearing
- Poor oral hygiene
- Raised temperature
- Frequent chest infections

When Eating or Drinking:

- Slow to initiate a swallow and / or delay in swallow (over 5 seconds)
- Uncoordinated chewing or swallowing
- Multiple swallows for each mouthful
- Pocketing of food in cheeks or throughout mouth
- Oral or nasal regurgitation of food / fluids
- Extended time to eat / drink
- Coughing or sneezing during / following eating

Following the consumption of food or drink:

- A wet hoarse sounding voice
- Fatigue
- Changes in respiratory pattern
- Regurgitating intake

Additional indicators of dysphagia:

Positioning/ Postural Factors

- Inability to achieve optimal positioning for safe swallowing (sitting fully upright, head in midline with chin tuck)
- Inability to sustain optimal positioning for safe swallowing (due to motor, cognitive or behavioural problems)
- Inability to remain seated fully upright for at least 30 minutes after food and fluid intake.

Alertness & Behavioural Factors

- Sedated / drowsy

- Unmotivated for oral intake / food refusal (check which consistencies are refused and why)
- Significantly agitated at mealtimes (i.e., vocalising / calling out during food or fluid intake)
- Over distractibility
- Poor response inhibition associated with compromised airway protection (talking or vocalising during food / fluid intake)
- Impaired orientation and memory ('forgetting' to swallow bolus)
- Poor impulse control and response inhibition (i.e., grabbing inappropriate food choice, or overly large boluses for swallowing capacity)
- Rule breaking behaviour (inappropriate for selection for swallowing function)
- Impaired new learning / ability to adopt compensatory strategies or adhere to staff prompts (i.e., rate reduction, over chewing bolus, supraglottal swallow technique)
- Impaired judgement of how much and how fast to consume food / fluid (tachyphagia, shovelling, overloading mouth / pharynx)
- Impaired judgement of appropriate food / fluid selection to achieve safe swallowing status

Oral preparatory / Oral transit / Pharyngeal / Oesophageal phase factors

- Difficulty managing own secretions : drooling
- Spillage of food / fluid from lips while chewing / swallowing
- Spillage of food / fluid posteriorly over base of tongue while chewing (evidenced by gagging or coughing before the swallow is initiated)
- Inefficient chewing / oral preparation of the bolus (due to (a) cognitive / behavioural factors / poor judgement (b) impaired oral motor control and / or (c) poor dentition)
- Effortful initiation of swallow
- Report of pain on swallowing
- Evidence of food / fluids residue in the mouth after swallowing
- Audible "clunking" swallow
- Coughing / choking during / after food / fluid intake
- Report of solids being 'stuck in throat'
- Spontaneous use of fluid to aid pharyngeal transit ('wash food down')
- Coughing on reclining / lying supine / bending over / changing posture (suggests pharyngeal pooling)
- Report of having to 'wash down' food
- Excessive burping / eructation
- Report of 'heartburn' or reflux
- Apparent deterioration in swallowing skills.

Upper Limb Motor Control Factors

- Difficulties manipulating unmodified utensils
- Inability to independently cut food into bite size pieces (takes overly large mouthfuls)
- Poorly controlled plate / cup to mouth transfer (haphazard / uncontrolled presentation of food / fluids to mouth)

Evidence on Food and Fluid Intake

General:

- Difficulties managing oral secretions or drooling
- Absence or weakness of a voluntary cough or swallow
- Changes in voice quality/ tone (hoarseness/ moist sounding)
- Decreased mouth and tongue movements
- Tongue thrust/ primitive oral reflexes
- Frequent throat clearing
- Poor oral hygiene
- Raised temperature
- Frequent chest infections

When Eating or Drinking:

- Slow to initiate a swallow and / or delay in swallow (over 5 seconds)
- Uncoordinated chewing or swallowing
- Multiple swallows for each mouthful
- Pocketing of food in cheeks or throughout mouth
- Oral or nasal regurgitation of food / fluids
- Extended time to eat / drink
- Coughing or sneezing during / following eating

Following the consumption of food or drink:

- A wet hoarse sounding voice
- Fatigue
- Changes in respiratory pattern
- Regurgitating intake

Additional indicators of dysphagia:

Positioning/ Postural Factors

- Inability to achieve optimal positioning for safe swallowing (sitting fully upright, head in midline with chin tuck)
- Inability to sustain optimal positioning for safe swallowing (due to motor, cognitive or behavioural problems)
- Inability to remain seated fully upright for at least 30 minutes after food and fluid intake.

Alertness & Behavioural Factors

- Sedated / drowsy
- Unmotivated for oral intake / food refusal (check which consistencies are refused and why)
- Significantly agitated at mealtimes (i.e., vocalising / calling out during food or fluid intake)
- Over distractibility

Appendix 3: PMR14B Dysphagia Swallow Screen (DSS) page 1

[illegible]

Appendix 3: PMR14B Dysphagia Swallow Screen (DSS) page 2

NORTH METROPOLITAN HEALTH SERVICE MENTAL HEALTH Hospital DYSPHAGIA SWALLOW SCREEN (DSS)		Surname		Sex	U.R. No
		Forenames		D.O.B.	
		Address			
		Ward	Registrar	Consultant	
		Use Patient I.D. Label when available			
INSTRUCTIONS FOR DYSPHAGIA SWALLOW SCREEN (DSS) STRICT NURSING OBSERVATION AT ALL MEALS Only to be used by nurses trained in the use of the Dysphagia Swallow Screen (PMR14b) Direct Observation of a patient drinking fluids and/or eating food <u>for every meal</u> . Record all YES responses on Dysphagia Swallow Screen e.g. If patient is unable to hold food in mouth record B1 on PMR14b					
IDENTIFIED RISKS					
No.	A – FLUIDS	Yes	No		
1	Unable to hold fluid in mouth				
2	Initiation of swallow is extremely slow				
3	Little observed laryngeal elevation				
4	Coughing / choking during or after drinking				
5	Unclear 'gurgly' voice quality after swallowing fluids				
No.	B – DIET	Yes	No		
1	Unable to hold food bolus in mouth				
2	Unable to chew food bolus				
3	Initiation of swallow is extremely slow				
4	Little observed laryngeal elevation				
5	Coughing / choking when eating				
6	Unclear 'gurgly' voice quality after eating				
7	Food remains in mouth after eating				
8	Patient complains of a sensation of obstruction whilst eating				
No.	C – BEHAVIOUR	Yes	No		
1	Very fast intake (tachyphagia)				
2	Overfilling mouth				
3	Eating non-food items (pica)				
4	Extreme distractibility e.g. constantly moving				

Appendix 4: Dysphagia Risk Alert Sticker

DYSPHAGIA ALERT

This patient has an identified
risk of choking.

Please see speech pathology report
before offering food or drink.

Appendix 5: Clinical Alert Form (PMR01) (Page 1)

[illegible]

Appendix 5: Clinical Alert Form (PMR01) (Page 2)

[illegible]

"To cancel / remove a Clinical Alert" cross with two diagonal lines, sign and date



Appendix 6: Speech Pathology Care Plan and Report (PMR14) (Page 1)

NORTH METROPOLITAN HEALTH SERVICE MENTAL HEALTH		Surname		Sex	U.R. No
Hospital SPEECH PATHOLOGY REPORT/NOTES		Forenames		D.O.B.	
		Address			
		Ward	Registrar	Consultant	
Use Patient I.D. Label when available					

**OLDER ADULT MENTAL HEALTH
SPEECH PATHOLOGY SWALLOW INITIAL ASSESSMENT SUMMARY**

MEDICAL DIAGNOSIS: _____

ORAL MUSCLE FUNCTION:				COUGH:			
Jaw	Open	Tongue	Around lips	Reflex Cough			Effective
CN V	Close	CNXII	Around gums				Ineffective
	Lateral R		Into cheek R				Prompt
	Lateral L		Into cheek L				Delayed
Lips	Purse	Palate	Symmetry				Absent
CN VII	Retract	CN IX, X, XI	Closure	Cued Cough			Effective
Cheeks	Tone	Dentition					Ineffective

Comments: _____

SWALLOW PHYSIOLOGY:	COMMENTS:
Impaired oral bolus preparation	
Impaired oral bolus formation	
Increased oral transit time	
Reduced oral control	
Reduced oral clearance	
Reduced soft palate closure	
Delayed swallow initiation	
Reduced laryngeal excursion	
Inco-ordinated laryngeal movement	
Pharyngeal residue	
Laryngeal penetration/aspiration	
Reduced oesophageal motility	
Reflux	
Regurgitation	

SAFETY FURTHER COMPROMISED BY:	
Reduced and fluctuating alertness	Reduced co-operation
Fatigue's rapidly	Fluctuating behavioural state
Impaired cognitive state / confusion	Poor positioning
Reduced concentration / distractibility	Impulsiveness / inappropriate rate
Impaired airway protection	Reduced awareness
Shortness of breath	Overfilling mouth
Dystonia – cervical / orofacial / laryngeal	Anxiety / agitation
Inadequate dentition	Involuntary movements
Food pocketing/holding	Unable to use compensatory techniques

SWALLOW DIAGNOSIS & SEVERITY: _____

ADDITIONAL COMMENTS: _____

SNR Speech Pathologist _____ **Date of Assessment** _____

Revised 2020

PMR 14 SPEECH PATHOLOGY REPORT/NOTES

Appendix: 6 Speech Pathology Care Plan and Report (Page 2)

NORTH METROPOLITAN HEALTH SERVICE MENTAL HEALTH Hospital		Surname		Sex	U.R. No
		Forenames		D.O.B.	
		Address			
		Ward	Registrar	Consultant	

SPEECH PATHOLOGY REPORT/NOTES

OLDER ADULT MENTAL HEALTH SWALLOWING CARE PLAN

(*For Food and Fluid Level explanations, please refer to The International Dysphagia Diet Standardisation Initiative www.iddsi.org)

FLUIDS*

☐ THIN (Normal). Level 0

☐ SLIGHTLY THICK. Level 1

☐ MILDY THICK. Level 2 (Level 150)

☐ MODERATELY THICK. Level 3 (Level 400)

☐ EXTREMELY THICK. Level 4 (Level 900)

FOODS*

☐ LIQUIDISED. Level 3

☐ PUREED. Level 4

☐ MINCE & MOIST. Level 5

☐ SOFT & BITE SIZED. Level 6

☐ EASY CHEW- NO BREAD. Level 7

☐ EASY CHEW + BREAD. Level 7

☐ REGULAR (As desired). Level 7

☐ Extra gravy / sauce

SPECIAL EQUIPMENT

☐ Dysphagia cup

☐ Nose cut-out cup

☐ Plate guard

☐ Scoop bowl

☐ Built up cutlery

☐ Non-slip mat

☐ Straw

INDEPENDENCE

☐ Independent

☐ Stand-by supervision

☐ One to one supervision

☐ Partial assist

☐ Full assist

REFERRAL TO

☐ Team for consideration of non-oral feeding

☐ Dietician for provision of nourishing supplements

☐ Dental clinic for denture review

☐ Videofluoroscopy clinic

SAFE SWALLOWING STRATEGIES

☐ Sitting upright and alert with head in midline

☐ Orient patient to meal

☐ Reduce distractions

☐ Encourage chin down posture

☐ Encourage slow eating rate

☐ Encourage small mouthfuls – tspn vs dspn

☐ Ensure each mouthful is swallowed

☐ Double swallows

☐ Alternate diet and fluids

☐ Encourage small sips from a cup

☐ Straw for drinking

☐ Offer only top ½ glass

☐ Small plate and teaspoon

☐ Discourage talking while eating

☐ Regular swallow-cough-re-swallow

☐ Cease oral intake if:

- drowsy
- coughing
- after 45 min
- wet/gurgly voice quality
- upper airway sounds
- fatigued or
- non-compliant

☐ Stay upright for approx 30 mins. after meals

☐ Complete mouth-care at end of all meals

☐ Medications in thickened fluid – LEVEL:

☐ Medications in yoghurt

☐ Crushed medications

*Please consult the Pharmacist on the safety of combining the medication with the above, and on the safety of crushing medications before doing so.

SEE ADDITIONAL STRATEGIES (see details on ward Care plan)

☐ Specific Positional strategies

☐ Behavioural Strategies

☐ Medication strategies

☐ Swallow exercises

☐ Water Protocol

☐ Other

SNR Speech Pathologist

Date of Assessment

PMR 14 SPEECH PATHOLOGY REPORT/NOTES

THIS CARE PLAN IS ACCURATE AT TIME OF ASSESSMENT, AND NO RESPONSIBILITY IS TAKEN FOR FLUCTUATING PATIENT STATUS. Revised 2020

Appendix 7: IDDSI International Framework

The IDDSI Framework

Providing a common terminology for describing food textures and drink thicknesses to improve safety for individuals with swallowing difficulties.



© The International Dysphagia Diet Standardisation Initiative 2019 @ <https://iddsi.org/framework/>

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Derivative works extending beyond language translation are NOT PERMITTED.

Appendix 8: National Safety and Quality Health (NSQHS) Standards v2

The following standards are relevant for Dysphagia Management: ([NSQHSS v.2](#))

- **Standard 1** – Action 1.20: The health service organisation uses its training systems to:
 - Assess the competency and training needs of its workforce
 - Implement a mandatory training program to meet its requirements arising from these standards
 - Provide access to training to meet its safety and quality training needs
 - Monitor the workforce's participation in training
- **Standard 1** – Action 1.27: The health service organisation has processes that:
 - Provide clinicians with ready access to best-practice guidelines, integrated care pathways, clinical pathways and decision support tools relevant to their clinical practice
 - Support clinicians to use the best available evidence, including relevant [clinical care standards](#) developed by the Australian Commission on Safety and Quality in Health Care
 - **Standard 2** – Action 2.4: The health service organisation ensures that its informed consent processes comply with legislation and best practice
- **Standard 5** – Action 5.10: Clinicians use relevant screening processes:
 - On presentation, during clinical examination and history taking, and when required during care
 - To identify cognitive, behavioural, mental and physical conditions, issues, and risks of harm
 - To identify social and other circumstances that may compound these risks
 - **Standard 5** – Action 5.11: Clinicians comprehensively assess the conditions and risks identified through the screening process
 - **Standard 5** – Action 5.12: Clinicians document the findings of the screening and clinical assessment processes, including any relevant alerts, in the healthcare record
 - **Standard 5** – Action 5.27: The health service organisation that admits patients overnight has systems for the preparation and distribution of food and fluids that include texture modified diet and fluids and nutrition care plans based on current evidence and best practice.
 - **Standard 5** – Action 5.28: The workforce uses the systems for preparation and distribution of food and fluids to:
 - Meet patients' nutritional needs and requirements
 - Diet, fluids and texture modification requirements
 - Monitor the nutritional care of patients at risk
 - Identify, and provide access to, nutritional support for patients who cannot meet their nutritional requirements with food alone
 - Support patients who require assistance with eating and drinking
- **Standard 6** – Action 6.3: Clinicians use organisational processes from the Partnering with Consumers Standard to effectively communicate with patients, carers and families during high-risk situations to:
 - Actively involve patients in their own care
 - Meet the patient's information needs
 - Share decision-making

